BRIGHTWATER HEALTH PLAN

Medical Policy: MRI of the Lumbar Spine

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1. Purpose and scope

This policy sets out when Brightwater Health Plan authorises an MRI scan of the lumbar spine before it is given, and what a requesting office must show.

It is used by the Intake Coordinator who opens a review, by the UM Nurse Reviewer who decides it, and by Provider Services Representatives answering requesting offices, as the Utilisation Management Policy describes.

Each service has its own medical policy. The five medical policies share one layout and many of the same sentences; they differ in the diagnoses and sites of section 2, in the coverage criteria of section 4, in how section 5 reads them, and in the documents, care and setting of sections 6, 8 and 9. A request is always reviewed under the policy of its own service, and a criterion in one policy never applies to a request for another service.

The policy applies to members of every Brightwater plan.

2. Service covered

This policy covers MRI of the lumbar spine, service code RAD-LSM, for these diagnoses:

- M54.50 - low back pain, unspecified
- M54.16 - radiculopathy, lumbar region
- M54.17 - radiculopathy, lumbosacral region
- M51.26 - other intervertebral disc displacement, lumbar region
- M48.061 - spinal stenosis, lumbar region, without neurogenic claudication
- M43.16 - spondylolisthesis, lumbar region

The service is given at an independent imaging centre or a hospital outpatient department.

A request for another service is reviewed under that service's own policy, never under this one.

3. How this policy is applied

When a request for the service arrives, the Intake Coordinator opens its review and the questionnaire is screened against the criteria of section 4, read as the Prior Authorisation Rules set out.

The criteria are read from this policy and from no other document, and the review records each criterion beside the sentence it was read from.

Each criterion is Met, Not met or Not documented on the questionnaire as received. The latest submission of a request is the request; an earlier submission is never screened.

The questionnaire decides every criterion. The clinical documents attached to the request are evidence for the reviewer, read with the evidence guides; a document never fills a blank answer and never replaces one.

The screening recommends one of the decisions open, and a UM Nurse Reviewer who did not open the review decides, as the Utilisation Management Policy describes.

4. Coverage criteria

Every criterion below must be met for a UM Nurse Reviewer to approve the service. Each criterion is stated once, and a request is screened against all of them:

1 **Minimum age.** The member is aged 18 years or over.

2 **Conservative treatment or red flag.** The member has completed at least 6 whole weeks of conservative treatment, or a red flag symptom is reported.

3 **Repeat imaging interval.** A scan of the same region less than 12 months ago is repeated only when new neurological symptoms are reported.

A criterion is never met by a figure in a letter, and never waived except as it says itself.

5. Reading the criteria

Conservative treatment is treatment of the back without surgery: physiotherapy, supervised or home exercise, manual therapy, pain and anti-inflammatory medicines and activity modification. Its length is the total time spent in conservative treatment, counting overlapping courses once, in whole weeks, as the Prior Authorisation Rules set out.

A red flag symptom is one of six: bowel or bladder dysfunction, saddle anaesthesia, progressive neurological deficit, suspected malignancy, suspected infection or significant trauma. A red flag must be reported on the questionnaire; one described only in a letter is evidence for the reviewer, as the evidence guide for red-flag symptoms sets out.

A scan of the same region is an MRI or CT of the lumbar spine. New neurological symptoms are weakness, numbness or a change in reflexes that began after that scan.

The minimum age is read from the member's age on the request. A request for a younger member is referred to a Physician Reviewer, who reviews it under the plan's paediatric guidance.

6. Documentation to send

The requesting office should send with the request the dates and length of each course of conservative treatment for the back, such as the physiotherapy discharge summary or an attendance record; the clinic letter describing the back and leg symptoms and any red flag symptom; and the report of any earlier MRI or CT of the lumbar spine, with how many months ago it was done. When the request repeats a recent scan, describe the new neurological symptoms and when they began.

7. When a criterion is not met

A request with a criterion that is not met is referred to a Physician Reviewer, and only a Physician Reviewer may deny a request. A UM Nurse Reviewer may also refer a request whose criteria are all met, when the clinical documents raise a concern the criteria do not cover.

The Physician Reviewer may approve a request that does not meet a criterion when the clinical documents show that the service is necessary for the member, and records why in their own words.

The requesting office may ask to discuss a referred request with the Physician Reviewer before a decision, in a peer-to-peer discussion.

A criterion that is not documented is never on its own a reason to refer a request: the missing answer is asked for first, and the request is referred when the answer does not arrive in time or shows a criterion not met, as the Utilisation Management Policy describes.

8. Care before this service

Most back pain improves with activity, simple pain relief and physiotherapy, and an MRI rarely changes treatment before a full course of conservative treatment has been tried. Plain X-rays are not needed for most back pain without a red flag. A member waiting for a scan should keep up the exercise programme their physiotherapist set.

9. Setting and length of stay

An approved scan is authorised once, as an outpatient, at an independent imaging centre or a hospital outpatient department. No inpatient days are authorised under this policy.

10. Questions about this policy

The Medical Policy Committee owns this policy, reviews it every twelve months and issues any change as a new issue of this document, replacing every earlier issue.

Questions about this policy from a requesting office go to Provider Services, which answers them from the medical policies, the Utilisation Management Policy, the Prior Authorisation Rules and the evidence guides. Questions from Brightwater staff go to the utilisation management team.